



POLICY

Confidentiality, Disclosure and Transmission of Health Information

Scope (Staff):	All staff
Scope (Area):	CAHS (CACH, CAMHS, PCH and Neonatology)

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

**This policy is currently under review.
The policy content remains current throughout
this review process.**

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Aim

To outline the governance and procedures pertaining to confidentiality of patient / client Health Information and the disclosure and transmission of this information to patients / clients, parents and legal guardians, or authorised external parties.

Risk

Breach of patient / client confidentiality and inappropriate release of Health Information, allegations of staff misconduct, disciplinary action, legal implications, and/or damage to health service reputation.

Definitions

Confidentiality: the treatment of information that an individual has disclosed in a relationship of trust and with the expectation that it will not be used or divulged to others in ways that are inconsistent with the understanding of the original disclosure, without permission.⁴

Duty of Confidentiality: obligation imposed on persons by common law, statute and /or equity which requires that information of a certain character (e.g. personal or otherwise sensitive information) be treated in confidence by those to whom it is made known or becomes known.¹

Health Information: means information (or an opinion) that is also personal information about the health or disability of an individual, an individual's expressed wishes regarding their healthcare, health services provided to an individual, or any personal information collected for the purpose of providing a health service (this would include patient or client medical or health care records).¹

Mature Minor (also known as child with capacity) in the context of disclosing their confidential information means: a person under the age of 18 who has the capacity to adequately understand and appreciate the reason for, and the consequences of, their confidential information being disclosed. A mature minor can consent to disclosure of their own health information to others.²

Non-Personal Information: information from which a person's identity is not apparent and cannot be reasonably ascertained.

Whether information is truly non-personal will depend on the context, including the nature of the information, the number of people to whom it could potentially relate, and the amount of information proposed to be disclosed.

- Although a series of individual pieces of information may not, on their own, enable the identity of an individual to be ascertained, identification may occur when all the pieces of information are combined together.²

Personal Information: information or an opinion, whether true or not, and whether recorded in a material form or not, about an individual, whether living or dead whose identity is apparent or can reasonably be ascertained from the information or opinion; or who can be identified by reference to an identification number or other identifying particular such as fingerprint, retina print or body sample.²

Privacy: the individual's right or expectation that health information and other identifying information will not be disclosed.⁴

Confidentiality of Health Information

- Patient and client Health Information is managed according to the [WA Health Information Access, Use and Disclosure Policy](#)¹ and the WA Health [Patient Confidentiality Policy](#).²
- All staff with access to Health Information as a part of their role have a duty to maintain the confidentiality of all information acquired, created or disclosed to them.
 - This duty is enduring and extends beyond the delivery of care, beyond the end of the professional client / patient relationship and beyond the death of the patient.¹
- Where a patient or client is referenced in an internal CAHS document, the patient or client unit medical record number (UMRN) and **not** the patient name must be used.
 - If a client does not have a UMRN, client name and date of birth is acceptable. To ensure patient / client confidentiality is maintained, documentation containing confidential patient / client information must be marked and stored according to the WA Health [Information Classification Policy](#).⁴

Confidentiality Breach

- Any breach of confidentiality may lead to disciplinary action, including the matter being treated as serious misconduct as per the WA Health [Discipline Policy](#).³
 - Observed breaches of confidentiality must be reported to the relevant line manager or head of department.
 - Where a breach of confidentiality in relation to Health Information occurs, refer to the CAHS [Data Breach Response](#) procedure.

Disclosure of Health Information

- As per the WA Health [Patient Confidentiality Policy](#)², exceptions to the duty of patient confidentiality include:
 - The patient or client has given their consent to the proposed disclosure

- For **PCH patients**, the [Consent for Release of Information CAHS](#) form must be completed by the parent, legal guardian or Mature Minor.
- For **CAHS Community Health clients**, refer to the Community Health [Consent for Release of Client Information](#) procedure.
- For **CAHS Mental Health patients**, refer to the section on the [Mental Health Act 2014](#) in [Appendix 1](#) of this policy.
- For further detail regarding consent, including the consent to disclosure principles for children and Mature Minors, refer to the WA Health [Patient Confidentiality Policy](#)².
- The law requires or permits the disclosure; and
 - Refer to [Appendix 1](#) for circumstances in which the disclosure of Health Information may be permitted or required by statute.
- There is a 'public interest exception' to the duty of confidentiality:
 - This is a very limited exception and should not be relied upon without first obtaining legal advice on the particular circumstances of the case.
- For further detail regarding each of the above exceptions, refer to the WA Health [Patient Confidentiality Policy](#) ².
- Health Information released for the purposes of research must abide by the WA Health [Research Governance Policy and Procedures](#)⁷.

Storage and Transmission of Health Information

- Health Information stored in hardcopy or electronically must be managed according to the WA Health [Information Security Policy](#).⁴
- WA health system USB devices can be purchased via an HFN-030 form.
 - The use of non-WA health system USB devices in the WA health system network is prohibited and a breach of security.
- Confidential or personal information (including digital images, downloaded patient data, commercially sensitive information etc.) must **NOT** be held on portable devices unless the device is protected by approved cryptography.
- Printers, facsimile machines and other devices that output confidential or personal information must be subject to security controls.
- Health Information transmitted to external parties must be in accordance with policies under the WA Health [Information and Communications Technology](#) policy framework.³
- Patient / client information must only be transmitted electronically via approved WA Health methods.
 - Refer to [Appendix 2](#) for the CAHS Transmission of Health Information procedure for all approved methods of transmission and associated processes.

My File eXchange (MyFX)

- My File eXchange (or MyFX) is available for ad-hoc use, and as an on demand self-registration-based service to send and receive large files, sensitive and confidential information both inside and outside of the WA health system network.
- For more information on how to use MyFX please read the [My File eXchange \(MyFX\) user guide](#).

My File Transfer (MyFT)

- My File Transfer (or MyFT) is available to enable regular use, and / or ongoing business process integrated file transfer requirements, to send and receive large files, sensitive and confidential information both inside and outside of the WA health system network.
 - It also can provide collaboration-based file sharing services between working groups both internal and external to the WA health system.
 - All relevant CAHS staff can access MyFT internally and outside of the WA Health network via the <https://myft.health.wa.gov.au/> website.
 - Note that a license fee applies to this use.
 - For more information on how to use MyFT please read the [My File Transfer \(MyFT\) User Guide](#).

Related CAHS Policies Regarding Confidentiality of Health Information

- For the release of Health Information to the media refer to the [CAHS Media Communications](#) policy.
- For the procedure for providing hospital medical records to other Health Services, refer to the [PCH Access to Medical Records Policy](#).
- For legal requests for clinical reports, refer to [Appendix 1](#).
- Refer to the following policies regarding storage and transmission of confidential health information on mobile electronic devices:
 - [CAHS Mobile Phones – Corporate](#) policy
 - [PCH Bring Your Own Device \(BYOD\)](#) policy
- For information pertaining to confidentiality of health information in the storage and transmission of media forums (photography, video or audio), refer to:
 - [CAHS Photography and Video / Audio Recording CLINICAL policy](#)
 - [CAHS Photography and Video / Audio Recording NON CLINICAL policy](#)
- For confidentiality pertaining to the use of social media, refer to the CAHS [Social Media](#) policy.

1. Information Access, Use and Disclosure Policy (WA Health Mandatory Policy MP0015/16)
2. Patient Confidentiality Policy (WA Health Mandatory Policy MP0010/16)
3. Discipline Policy (WA Health Mandatory Policy MP0040/16)
4. Information Classification Policy (WA Health Operational Directive OD0537/14)
5. Information Security Policy (WA Health Mandatory Policy MP0067/17)
6. Information and Communications Technology (WA Health Policy Framework)
7. Research Governance Policy and Procedures (WA Health OD0411/12)
8. Mental Health Act 2014 (Western Australian Legislation)
9. Clinician's Practice Guide to the Mental Health Act 2014 (WA Chief Psychiatrist)
10. Chief Psychiatrist's WA Standards for Clinical Care 2015
11. Children and Community Services Act 2004 (Western Australian Legislation)
12. Guidelines for the Transmission of Personal Health Information by Facsimile Machine (WA Health Information Circular IC0179/14)
13. My Health Record (MHR) Policy (WA Health Mandatory Policy MP0091/18)
14. WA Health System Language Services Policy (WA Health Mandatory Policy MP0051/17)
15. Information Classification Policy (WA Health Operational Directive OD0537/14)
16. Prisons Act 1981 (Western Australian Legislation)
17. Criminal Law (Mentally Impaired Accused) Act 1996 (Western Australian Legislation)
18. Freedom of Information Act 1982 (Australian Federal Register of Legislation)
19. Guidelines for Protecting Children 2015 (WA Department of Health)
20. Health Services Act 2016 (Western Australian Legislation)
21. Parliamentary Commissioner Act 1971 (Western Australian Legislation)
22. Royal Commissions Act 1968 (Western Australian Legislation)
23. Health Services (Information) Regulations 2017 (Western Australian Legislation)

Related CAHS internal policies, procedures and guidelines

Public Interest Disclosure (CAHS Policy Manual)
Media Communications (CAHS Policy Manual)
PCH Access to Medical Records Policy (PCH Operational Manual)
Mobile Telephones – Corporate (CAHS Policy Manual)
Bring Your Own Device (BYOD) (PCH Operational Manual)
Photography and Video / Audio Recording CLINICAL (CAHS Policy Manual)
Photography and Video / Audio Recording NON CLINICAL (CAHS Policy Manual)
Social Media (CAHS Policy Manual)
Consent (CAMHS Policy Manual)
Telehealth Service (CAHS Policy Manual)
Data Breach Response (CAHS Policy Manual)

Useful resources (including related forms)
WA Health Legal and Legislative Services Unit
CAHS Application for Access to Health Information Form
Consent for Release of Information CAHS
Healthy WA - Access my WA Health Medical Records
Working With Youth - A Legal Resource For Community Based Health Workers
My File Transfer (MyFT) User Guide (WA Health Support Services)
Mandatory reporting of child sexual abuse (WA Health Statewide Protection of Children Coordination (SPOCC) Unit)

This document can be made available in alternative formats on request.

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Appendix 1: Disclosure by Operation of the Law

- The following are some examples of disclosures that are authorised (either permitted or required) by Law. This is not intended to be an exhaustive list of the types of disclosures that may be authorised by law, nor is it intended to provide staff with legal advice as to whether a particular disclosure is authorised.
 - Any questions or requests for legal advice on information disclosure should be directed to the Manager of Mediation and Legal Support Services (MLSS) at CAHS.MLSS@health.wa.gov.au; CAHS.ROI@health.wa.gov.au; 6456 4355 or 6546 4354.

FOI Applications and Administrative Release of Information

- A patient or client, parent or legal guardian or an authorised third party may apply for access to their personal health information under the [Freedom of Information Act 1992](#) (FOI Act)¹⁵ or request access to their personal health information outside the [FOI Act](#)¹⁵ by way of an Administrative Release (AR).
- FOI applications may also be received for non-personal information such as information not relating to the care and treatment of a particular patient (i.e. media requests for statistics).
- Applications made under the FOI Act must comply with the [FOI Act](#)¹⁵ and must be completed within 45 days unless negotiated otherwise.
- FOI or AR applications must be in writing or via a [CAHS Application for Access to Health Information Form](#) (available to the public via the PCH website) and be directed to the ROI Department, MLSS at:
 - CAHS.ROI@health.wa.gov.au; or
 - **Release of Information**
Child and Adolescent Health Service
Perth Children's Hospital
Locked Bag 2010
NEDLANDS WA 6909
- All FOI or AR applications must be accompanied by valid consent and certified identification dated within 12 months.

Health Services Act 2016

- The WA [Health Services Act 2016](#)²⁰ provides the collection, use or disclosure of information is authorised if the information is collected, used or disclosed in good faith in any of the circumstances outlined in section 220 of the Health Services Act.
 - Refer to the [Health Services Act 2016](#)²⁰ (section 220).
 - Refer to the [Health Services \(Information\) Regulations 2017](#) for the prescribed circumstances in which the collection, use or disclosure of information is authorised pursuant to section 220(1) (i) of the [Health Services Act 2016](#)²⁰.

Public Interest Disclosure Act 2003

- Refer to the CAHS [Public Interest Disclosure](#) policy.

Subpoena or Witness Summons

- A subpoena or witness summons is a legally binding document issued by a court requiring a person to attend court to give oral evidence and / or to produce documents to a court. Failure to comply with a subpoena or witness summons is a serious offence.
- Subpoenas and witness summons should be directed to the CAHS.MLSS@health.wa.gov.au for processing.

Orders to Produce / Search Warrant from the WA Police

- The West Australian Police (WAPOL) may seek access to health information.
- WAPOL must be directed to the [MLSS Release of Information](#)
- In order for CAHS to provide information without a valid consent an Order to Produce (OTP) or search warrant is required.
- All OTP's must be sent to MLSS ROI to action.
 - If an unsigned OTP is provided by WAPOL, MLSS ROI staff will liaise with WAPOL to seek validation of the OTP.
- Once the information is ready for release, WAPOL must either attend MLSS to collect the information or provide a WAPOL email address for electronic transfer of the information.
- The original signed OTP must be sighted and copied by MLSS ROI prior to the release of any documents.
- OTP's must be processed within the timeframe stipulated unless negotiated otherwise.

Coroner

- Please refer to the CAHS [Coronial Matters](#) policy.

Mental Health Act 2014

- Under sections 248-251 of the [WA Mental Health Act 2014](#)⁸, (MHA) a person who is or was provided with treatment or care by a mental health service is entitled to inspect and be given a copy of any relevant document relating to the person in the possession or control of the mental health service (section 248).
 - This right is subject to the restrictions on access in s 249 of the MHA.
 - This right is distinct from the right of persons to apply for information under the FOI Act and should be dealt with separately from an FOI application.
- Child and Adolescent Mental Health Service (CAMHS) staff must deal with any request made under the MHA to inspect or be given a copy of the medical record or other relevant document relating to the person as soon as practicable after the request is received.

- All requests made under the MHA must be directed to CAMHS staff.
- The Psychiatrist can refuse the request if they reasonably believe (section 249 MHA) that:
 - disclosing the information poses a significant risk to the health or safety of the person or to the safety of another person, or poses a significant risk of serious harm, to the person or another person; or
 - it would involve disclosing personal information about somebody else, or the information is of a confidential nature obtained in confidence, unless the personal / confidential information is about an individual who consents to the disclosure; or
 - the person is a mentally impaired accused required under the [Criminal Law \(Mentally Impaired Accused\) Act 1996](#)¹⁹ to be detained in an authorised hospital and the relevant document came into existence under, or for the purposes of, the [Prisons Act 1981](#).¹⁸
- If the request is refused (section 250 MHA):
 - the reasons must be documented in the medical record and a copy of the reasons given to the Mature Minor or if a child, the parent / legal guardian
 - S 248(3) (b) also requires that a record of the approved form is filed.
 - the Mature Minor or parent/legal guardian may nominate a medical practitioner and / or legal practitioner to inspect and be given a copy of the documentation
 - CAMHS must ensure that the documentation is provided to the nominated person as soon as practicable.
 - The nominated person cannot disclose or provide this information to the person being refused access (financial penalties apply to the nominated person if this provision is breached - [WA Mental Health Act 2014](#)¹⁰ section 251).
- Requests for access to personal information under the MH Act can be received by any CAMHS staff member.
 - A Psychiatrist from CAMHS must review the information or record, to determine any restrictions, prior to granting access.
 - The Service Manager (in Community and Specialised CAMHS Services) or Clinical Nurse Coordinator (in Acute CAMHS Services) is responsible for ensuring that the request is completed.
- CAMHS staff should refer to the [Clinician's Practice Guide to the Mental Health Act 2014](#)⁹ and the [Chief Psychiatrist's WA Standards for Clinical Care 2015](#)¹⁰ for further information on the right to access personal medical records.
 - The CAMHS [Consent Procedure](#) provides guidance on patient / client consent to the release of information to other parties, including to the Mental Health Advocacy Service to enable that Service to fulfil its functions under the MHA.

Children and Community Services Act

- The Department of Communities (DOC) may seek access to information from CAHS without parental consent under the Section 23(3) of the WA [Children and Community Services Act 2004](#) ¹¹ provided that it is relevant to the wellbeing of the child or otherwise meets the definition of 'relevant information' in that section.
- Section 23 applications from DOC for information from CAHS must be directed to [MLSS ROI Department](#).
- The application can only be made by an authorised officer from DOC and must be accompanied by a copy of the officers' staff identification badge.
- Only information that is deemed relevant to the wellbeing of a child should be released.

Mandatory Reporting of Child Sexual Abuse

- The [Guidelines for Protecting Children 2015](#) Section 2.3.2 outlines the procedures all WA Health doctors, nurses and midwives must follow in order to comply with the mandatory reporting requirements for child sexual abuse.

Australian Health Practitioners Regulation Agency (AHPRA)

- In accordance with the responsibilities for managing notifications received regarding registered health practitioners or students, the agency may seek to obtain a copy of health information to assist the board in making a decision on a matter.
- Applications from AHPRA are to be directed to the [MLSS ROI Department](#).

Criminal Injuries Compensation Act 2003

- The Department of Justice may seek access to information in relation to criminal injuries compensation investigations under the Criminal Injuries Compensation Act 2003, Section 19.
- CIC applications from Department of Justice seeking information from CAHS must be directed to [MLSS ROI Department](#).

Ombudsman

- The [Parliamentary Commissioner Act 1971](#) ²¹ and the [Royal Commissions Act 1968](#) ²² provides for the Ombudsman of Western Australia to order production of information in relation to an investigation that has been initiated of any decision or recommendation made, or any act done or omitted that relates to the matter being investigated.
- All Ombudsman applications must be directed to the CAHS Executive Office of the Chief Executive who will distribute as appropriate.
 - Generic email address - CAHSExecutiveOfficeofCE@health.wa.gov.au.

Appendix 2: Transmission of Health Information Procedure

- Once authorisation is given to provide information to third parties, there are various modes of transmission of information:

Sent by mail

- This is mainly for non-urgent information (anything not required immediately and by a specified date). This could be in hardcopy or in a form of a disc.
 - Care must be taken before sending photocopies of previous correspondence. Photocopies of letters from outside doctors, other hospitals or institutions must not be forwarded without the consent of the original doctor or institution.
 - Where documents are marked “Not for copy without the consent of the author” this must be observed.
 - If the information in the letters is important, the agency requesting the information must be notified if the other doctor or institution is prepared to write a report.

Facsimile transmission

- To be used only in an emergency situation where other types of transmission are not readily accessible and may hinder the speed of the information transfer.
- When a request is received for information to be sent via fax, the person receiving the request must record the following (usually on the fax cover sheet):
 - The name of the authority of the person making the request.
 - The name of the institution requesting the information (fax and telephone numbers).
 - The urgency of the request.
 - Details of the required information, including as much demographic data about the patient as necessary to identify and retrieve their medical record.
- When these details have been obtained, the person requesting the information is to be advised of the name of the person who will be processing the request and that they will fax the information as soon as possible.
- A CAHS fax cover sheet correctly completed must be the first page forwarded with **CONFIDENTIAL** and a confidentiality notice indicating that the information is confidential and limiting its use.
- The sender is responsible for ensuring the correct fax number has been dialled.
 - Fax machines which store frequently used numbers using “speed dial” should be used to assist in prevention of incorrect number dialling.
- The sender must ensure the recipient is expecting the fax prior to sending.
 - It is preferable that the receiver is standing by the fax machine when the information is transmitted.
- The recipient of the fax, at the conclusion of transmission, must telephone the sender to confirm receipt of the fax in its entirety.

- Verification of fax sent to correct number must be filed with the request paperwork upon completion.
- When receiving a fax where light sensitive paper is used, the faxed document must be photocopied and all copies filed in the medical/health record, including the fax cover sheet.
 - If an original document is forwarded at a later time, this must replace the photocopy in the health / medical record.
- Overseas requests for facsimile transmission of patient information require approval from the Chief Executive, CAHS, and may only be faxed from Executive Services.
- Confidential information being transmitted from other facilities to CAHS must be directed to the actual department requesting the information.
 - The main hospital fax number must only be used in cases where the requesting department does not have its own fax machine.
- The original document must be retained in the health / medical record in addition to the fax cover sheet listing the documents which have been forwarded and the fax verification document.
 - If many pages have been copied to assist the faxing of information, it is sufficient to list these items on the covering fax and file it in the Correspondence section of the health/ medical record.
 - For further information on transmission of patient information via fax, refer to the [WA Health Guidelines for the Transmission of Personal Health Information by Facsimile Machine](#).¹²

Email

- Health information must not be sent by email outside of the WA Health system network unless the information is encrypted or transferred via a secure file transfer (My FX or My FT) unless otherwise approved by an authorised officer as per the WA Health [Information Security Policy](#).^{5,9}

Phone

- Confidential patient / client information transmitted over the phone must be managed in line with the [Information Classification Policy](#).⁴ see page 10 Release – spoken word), which includes active measures to prevent unauthorised persons from overhearing / obtaining confidential information.
- Confidential patient / client information must not be left on home answering machines or other shared answering machines.

Secure Services

- Confidential patient information must be transmitted as per the WA Health [Information Security Policy](#).⁵
 - WA Health approved secure services include Remote Access, Secure File Transfer and WA Health approved secured USB.